

Class	Prototype	Action	Indication	Interactions	Contraindication	SE	AE	Education
Corticosteroids	Prednisone Dexamethazone	Suppress cytokines and prostaglandins Decrease swelling, pain, itch	Decrease Inflammation Immunosuppressant	Efficiency of Hypoglycemia drugs Anticoagulants	Systemic Fungal Infection HTN Hyperglycemia	Increase Blood Glucose GI Fluid Retention	Blood Loss Gastritis/Peptic Ulcer Cushingoid Appearance INFECTION RISK	Infection Prevention Take with Food No Live Vaccines DONT STOP ABRUPTLY, TAPER! Long-term use: Osteoporosis risk
Calcineurin Inhibitors	Cyclosporine	Suppress T-Cell Activity	Prevent Organ Rejection			Gum Hyperplasia Flu-Like Tremor	Infection Hepatotoxicity Nephrotoxicity	Taken for Life Infection Prevention
DMARDs/TNF Blockers	Methotrexate	Immunosuppression	RA Gold Standard		Pregnancy/Lactation Active infection	Nausea Fatigue Immunosuppression	Myelosuppression Hepatotoxicity	Taken Once/week Takes Longer to work: 3-6 months No Alcohol
DMARDs/TNF Blockers	Hydroxychloroquine	Immunosuppression	Malaria Lupus RA				Retinopathy	Regular Eye Exams
Biologic DMARDs	Adalimumab		RA Crohn's Disease Ulcerative Colitis		Recent/Reoccurring infection		INFECTION RISK Black Box: Malignancies	SQ Injection Used with conventional DMARD Tb Test before treatment No Live Vaccines
NSAIDs 1st Gen	Ibuprofen Aspirin	COX I & II Inhibitor Aspirin: Anti-platelet				heartburn nausea vomiting GI	Tinnitus (toxicity) Nephrotoxicity Reye's Syndrome in Children GI Bleed	
NSAIDs 2nd Gen	Celecoxib	COX II Inhibitor Only					Cardiac/Cerebrovascular Events	
Medications for Inflammation/Pain	Acetaminophen	Blocks Prostaglandin production	Pain and Fever				Hepatotoxicity	No more than 4g/day
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Alkylating Agent/ Nitrogen Mustard	Cyclophosphamide				myelosuppression		myelosuppression Hemorrhagic Cystitis Extravasation risk	**HYDRATE**
Anthracyclines	Doxorubicin hydrochloride					fatigue nausea myelosuppression stomatitis	Cardiotoxicity	Red-orange body fluids 1-2 days after transfusion
Mitotic Inhibitors	Vincristine	M Phase Specific					Neurotoxicity Peripheral Neuropathy	
Hormonal Therapy/ Estrogen Receptor Blocker	Tamoxifen			Estrogen-dependent breast cancer	Patients with DVT or PE	hot flashes vaginal dryness menstrual irregularities	uterine cancer risk blood clots	
Biologic Therapy/ Monoclonal Antibodies	Trastuzumab	Genetically engineered targeted therapy	Cancer Auto-immune disease Inflammatory Diseases	HER2 positive breast cancer	pregnancy	Fatigue Flu-like	Cardiotoxicity	Infection Prevention
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Tetracyclines	Tetracycline	Inhibits protein synthesis	Broad Spectrum Coverage		Pregnancy: 2nd/3rd Trimester	Teeth Discoloration Photosensitivity	Children under 8: Permanent Teeth Discoloration	Dairy/Iron reduce absorption
Bacteriostatic Agents: Lincosamides	Clindamycin	Inhibits Protein Synthesis	Skin and soft tissue infection				C-DIFF	
Bacteriostatic Agents: Sulfonamide Derivatives	Sulfamethoxazole-trimethoprim (TMP-SMX)	Disrupts Folic Acid Pathway	First Line: Uncomplicated UTI			GI	Steven Johnson Syndrome	Look For new rash (SJS)
Bactericidal Agents: Aminoglycosides	Gentamycin	Inhibits Protein Synthesis	Sepsis		Preexisting Kidney Impairment		Ototoxicity Nephrotoxicity	Report tinnitus, hearing changes, dizziness, vertigo
Bacteriostatic Agents: Beta-Lactams	Penicillins Cephalosporins	Inhibits cell wall synthesis	Many Common Infections	Penicillin cross-sensitivity with cephalosporins		GI	Hypersensitivity Risk Anaphylaxis	

Bacteriacidal Agents: Fluoroquinolones	Ciprofloxacin	Target bacterial DNA replication	Complicated UTI			GI	Tendonitis/ Tendon rupture	Avoid dairy, antacids within 2 hrs of dosing
Bactericidal Agents: Glycopeptides	Vancomycin	Weakens Cell Wall	MRSA Complicated Infections		Caution with renal impairment	GI	Nephrotoxicity Ototoxicity Vancomycin Flushing Syndrome	
Antitubercular Agents: Rifamycins	Rifampin	Blocks bacterial enzyme that makes RNA	Tb Treatment	Numerous, specifically oral contraceptives		Orange-red body fluids	Hepatotoxicity	Take as prescribed Complete full course
Antifungals: Azole Derivatives	Flucanazole	Interrupts Cell Wall Integrity of fungi	Vaginal Yeast Infection Oral Thrush	Can increase warfarin levels		GI	Hepatotoxicity QT Interval Prolongation	Take antacids one hour before, or two hours after Avoid alcohol
Antifungals: Polyene Macrolides	Amphotericin B	Disrupts Cell Membrane	Life Threatening Fungal Infections			Infusion Reaction Hypokalemia Hypomagnesemia	Nephrotoxicity Severe electrolyte Imbalance Black Box: Cardiac Arrest	
Antiparasitics: Antiprotozoal Agents	Metronidazole	Damages DNA in susceptible organisms	C-Diff diarrhea Anaerobic bacteria Infection Certain Parasites		Alcohol within 72 hrs	Nausea Metallic taste in mouth	Peripheral Neuropathy	
Antivirals: Medications to treat Influenza	Oseltamivir		Influenza A & B					Most effective early in illness (First 48 hrs)
Medications to treat HIV	Zidovudine	Interrupts DNA chain of HIV virus	HIV as part of combination treatment regimen					Adherence Taken for life
Medications to treat Herpes Virus	Acyclovir sodium	Interferes with DNA Replication *Does not eliminate herpes	Treat or Suppress Herpes outbreaks Infections caused by Herpes Simplex Virus			GI	Nephrotoxicity (especially when given IV)	